Informed consent

Ethical foundations, decision-making capacity, representation and refusal in Bulgarian law — with twelve cases

Practical Class 2 · Social Medicine and Medical Ethics I · 2026/2027

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Plan of the class

Block	Cases
1 · Concept and ethical foundations	—
2 · Information, form and scope	1–4 (plenary)
3 · Capacity, minors and representation	5–8 (plenary)
4 · Refusal, emergency, treatment against the will	9–12 (small groups)
5 · Practice: documentation and responsibility	—

How we work

- each case: facts → questions → your decision → **key points**
- always ask: **who decides, on what information, how freely, for which intervention, and under which article?**
- Bulgarian law is the reference; foreign rules are marked *comparative*; quotations of Bulgarian acts are working English translations

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1. **Concept and ethical foundations**
2. Information, form and scope
3. Capacity, minors and representation
4. Refusal, emergency and treatment against the will
5. Practice: documentation and responsibility

What informed consent is

Informed consent is the voluntary decision of a person with decision-making capacity to accept a specific medical intervention, after receiving and understanding information about its nature, purpose, benefits, risks and alternatives. It is a **process of communication**, not a signature.

> *Medical activities shall be carried out after informed consent has been expressed by the patient.*
>
> — Health Act (Bulgaria), Art. 87(1)

> *An intervention in the health field may only be carried out after the person concerned has given free and informed consent to it. ... The person concerned may freely withdraw consent at any time.*
>
> — Oviedo Convention, Art. 5

Milestones: Schloendorff v. Society of New York Hospital (USA, 1914) — self-determination over one's body · Nuremberg Code (1947) — voluntary consent in research · Salgo v. Leland Stanford (USA, 1957) — the term "informed consent" · Oviedo (1997) — binding treaty; for Bulgaria in force since 01.08.2003.

Why consent matters: four ethical foundations

Autonomy

the person decides what happens to his body and life — even when the choice differs from the doctor's recommendation

Well-being

only the patient can judge how a benefit or risk fits his values, work, family and fears

Trust

honest, unpressured communication supports trust and the disclosure on which good care depends

Dignity and integrity

touching or operating without permission treats the person as an object — merely as a means

Paternalism

the physician decides "for the patient's good". **Soft** paternalism protects someone whose choice is not autonomous; **hard** paternalism overrides an autonomous choice — almost never justified today.

Shared decision-making

the clinician brings evidence, options and uncertainty; the patient brings goals and values. **Consent authorises** an offered intervention; it does not oblige the physician to provide clinically inappropriate treatment.

Five elements of valid consent

Element	Practical question	Sign of a defect
Sufficient information	Does the patient know what is proposed, why, and what the realistic alternatives are?	the decision rests on an omitted or distorted material fact
Understanding	Can the patient explain in his own words the main benefit, risk and consequence of refusal?	repeats the words of the form without grasping what they mean for him
Decision-making capacity	Can he weigh the information against his own situation and express a stable choice?	an acute cognitive or mental problem impairs judgement on this question
Voluntariness	Is there a real possibility to accept or refuse without threat or undue pressure?	fear, dependence, deception or manipulation
Specific expression of will	For which intervention, to what extent and under which conditions?	a general signature used for unforeseen or different interventions

Textbook of medical ethics, ch. 6 (table of the elements of valid informed consent)

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What the treating physician must explain

To obtain informed consent, the treating physician (dentist) informs the patient — respectively the parent, guardian, curator or other person under Art. 87 — about:

1. *the diagnosis and the nature of the disease;*
2. *the aims and nature of the treatment, **reasonable alternatives**, expected results and prognosis;*
3. *the potential risks of the proposed methods, including side effects, adverse drug reactions, pain and other discomfort;*
4. *the probability of a favourable effect and the risk to health with other methods **or with refusal of treatment**.*

— Health Act, Art. 88(1) (abridged)

*The information is provided **in good time and in an appropriate volume and form**, enabling freedom of choice of treatment.*

— Health Act, Art. 88(2)

Comparative: how much to disclose?

professional standard (what doctors usually tell) · *reasonable patient* standard (what a reasonable person would want to know) · *subjective* standard (what this patient needs). UK: Montgomery v Lanarkshire Health Board [2015] UKSC 11 — material risks.

Written form and the right not to know

> *For surgical interventions, general anaesthesia, invasive and other diagnostic and therapeutic methods that lead to an increased risk to the patient's life and health or to a temporary change of consciousness, the information under Art. 88 and the informed consent are provided* ***in writing****.*
>
> — Health Act, Art. 89(1)

The form documents the conversation; it does not replace it.

Forms of consent: **express** (oral or written) or **implied** by conduct for simple, low-risk acts (cooperating with a blood-pressure measurement after explanation); silence or lack of resistance is not consent. Admission to hospital is not blanket consent; emergency authority under Art. 89(2) is not "implied consent".

Health Act, Art. 92: the treating physician informs the patient about his state, the disease and prognosis, the planned activities and their risks, the alternatives, and the **names, positions and specialties** of the team.

- The patient may **decline** information about the disease, prognosis and planned activities — except where his condition endangers the health of others (Art. 92(2)); this is **recorded in writing** (Art. 92(3)).
- He may authorise **in writing** a person to be informed instead of him (Art. 92(4)) — this person receives information; he does not thereby decide.

Scope of consent and checking understanding

Scope

- consent authorises a **defined** intervention — not "whatever proves necessary";
- **foreseeable extensions** (e.g. conversion to open surgery) are discussed and written in advance;
- a materially different intervention requires renewed information and consent — in writing where Art. 89(1) requires it;
- consent may be **withdrawn at any time** (Oviedo, Art. 5); an intervention under way is stopped or modified safely and the consequences explained.

Understanding

- plain language and natural frequencies with the same denominator: a risk falling from 6 to 3 in 100 is a **3-point absolute** and a **50% relative** reduction (hypothetical);
- **teach-back**: "Tell me in your own words what we will do and what can go wrong" — it checks the doctor's explanation, not the patient;
- time, interpreter, written material, a person the patient trusts.

Consent is not a waiver

A disclosed complication of a properly indicated and competently performed intervention does not in itself establish negligence; disclosure never excuses negligent care, a breach of standards or a concealed error.

Case 1 · The obliging nurse

The case

A surgeon, busy with other patients, phones the ward nurse and asks her to "get the written consent" for tomorrow's operation. The nurse hands the form to the patient, who signs without reading it. After surgery a wound infection develops — a complication listed in the form, but never explained to the patient by the treating physician. The patient files a complaint that he was not informed.

1. Is the complaint well-founded?
2. May the nurse obtain the consent? What was her correct role?
3. What does the signature prove — and what does it not prove?

Case 1 · The obliging nurse — key points

Law

- The duty to inform lies with the **treating physician** (Health Act, Art. 88(1)); for surgery both the information and the consent are **written** (Art. 89(1)).
- A signature without explanation proves that a form was signed, not that the patient was **informed** — the complaint about information is well-founded.
- A complaint can go to the Regional Health Inspectorate (Art. 93) and, for an ethics breach, to the BMA regional college.

Ethics and practice

- The nurse may **support** the process — deliver the form, answer practical questions, tell the doctor that the patient has questions — but cannot replace the physician's conversation.
- The infection itself is not proof of fault: a known complication of correctly performed surgery does not establish negligence; the problem here is the missing explanation.
- Correct action: the surgeon (or a team physician) explains before surgery; the nurse refuses to "collect a signature" alone.

Case 2 · The alternative not mentioned

The case

A man with newly diagnosed chronic-phase chronic myeloid leukaemia is offered allogeneic stem-cell transplantation and is fully informed about its risks, including graft-versus-host disease (GVHD). After transplantation he develops severe GVHD. He then learns that oral tyrosine-kinase inhibitors (e.g. imatinib) are the standard first-line treatment, control the disease in most patients and, in selected patients with a sustained deep molecular response, allow planned discontinuation under molecular monitoring — and that transplantation is usually reserved for disease not controlled by them. Nobody mentioned this. He complains.

1. Was his consent informed, although every risk of transplantation was explained?
2. Which item of Health Act Art. 88(1) is breached?
3. Why was transplantation proposed at all? What would he have to show to obtain damages?

Case 2 · The alternative not mentioned — key points

Law

- Art. 88(1)(2) requires information about **reasonable alternatives**; the standard first-line treatment is plainly one. Consent was **not informed**.
- Explaining all risks of one option does not cure the omission of another; whether the proposal was clinically justified at all is a separate question.
- For damages the patient must also prove harm and a causal link to the breach (Obligations and Contracts Act — contract or tort; Class 4).

Ethics

- Options differ in benefit, burden, uncertainty and chance of treatment-free remission; weighing them is a value choice that belongs to the patient.
- Omitting an alternative because the doctor prefers the other option is covert paternalism.
- Good practice: present options side by side — benefit, risk, burden, uncertainty — and record which ones were discussed.

Case 3 · The informed risk

The case

A man needs revascularisation of a coronary artery. He is told that angioplasty carries a small risk of vessel rupture and fatal bleeding and that bypass surgery is an alternative. He understands, asks questions, and chooses angioplasty in writing. The procedure is performed according to good practice, but the artery ruptures and he dies. The family complains that "he should never have been allowed to take that risk".

1. Is there a breach of the consent rules?
2. What did the patient accept by consenting — and what not?
3. When would the outcome of the complaint be different?

Case 3 · The informed risk — key points

Law

- Provided that expected benefit, prognosis and the trade-offs of the alternative were also discussed (Art. 88(1)(2)–(4)), the form was written (Art. 89(1)) and the choice voluntary, **consent was valid**.
- The realised risk was **disclosed and accepted**; a complication does not in itself establish a breach.
- The outcome changes if the procedure was **performed negligently**: consent never covers negligence.

Ethics

- Respect for autonomy includes the right to accept a risk the family would not accept.
- The family's grief deserves time and an honest explanation of what happened (disclosure after an adverse event).
- Documentation of the conversation protects both the patient's choice and the team.

Case 4 · Decisions under anaesthesia

The case

A 40-year-old man signs written consent for removal of nasal polyps. During the operation the surgeons find a suspicious lesion on a nasal turbinate; frozen section shows early cancer. Removing the turbinate may prove adequate treatment, but staging and the definitive plan need further work-up. Doing it now would spare him a second general anaesthetic; his wife is in the waiting room.

1. May the surgeons extend the operation without new consent?
2. Can the wife consent instead of him?
3. How could this dilemma have been prevented?

Case 4 · Decisions under anaesthesia — key points

Law

- Consent covers the **polypectomy only**. Art. 89(2) allows a written-consent-free intervention only when **life is immediately threatened** — an early cancer is not an emergency.
- The wife has **no legal authority** to give consent for an adult who will regain capacity within hours (Art. 87 lists the persons who may consent; a spouse is not among them). The word "relatives" in Code Art. 26 creates no representative authority.
- Correct course: complete the polypectomy; staging and treatment planning can safely wait; inform him and obtain new written consent.

Ethics and prevention

- "Sparing a second anaesthetic" is a real benefit, but it is the **patient's** trade-off to make.
- Prevention: discuss **foreseeable findings** before surgery and record what the patient authorises ("if a tumour is found, I agree / do not agree to …").
- Code of Professional Ethics, Art. 26: no procedure without consent except for an immediate threat to life.

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Two different concepts: legal capacity and decision-making capacity

Capacity to act (*deesposobnost*) — a status set by law

Persons and Family Act (distinct from the capacity to hold rights, *pravosposobnost*, acquired at birth, Art. 1):

- under 14 — **child under 14** (*maloleten*): acts through a parent or guardian (*nastoynik*) (Art. 3);
- from 14 to under 18 — **minor** (*nepalnoleten*): acts with the consent of a parent or curator (*popechitel*) (Art. 4);
- from 18 — adult with full capacity to act (Art. 2);
- **interdiction**, full or limited, only by court decision (Art. 5).

Decision-making capacity — a clinical judgement

For **this decision, at this time**. A diagnosis, old age, low education, an unusual choice or disagreement with the doctor do **not** by themselves prove incapacity. Capacity can be temporarily lost (delirium, hypoxia, sedation) and **regained**.

A formally capable adult may be temporarily unable to decide; a person with a psychiatric diagnosis may be fully able to decide. Medicine assesses capacity; interdiction and its lifting are decided by a court — a diagnosis by itself does not change legal capacity.

Assessing decision-making capacity

Understand

can he state in his own words what is proposed and why?

Appreciate

can he recognise how the condition and the options may affect **him**?

Reason

can he compare options and consequences consistently with his values?

Express a choice

can he communicate a choice? Explore fluctuations and contradictions

Proportionality: the more serious and irreversible the consequences, the more carefully capacity is established — but a refusal is not in itself evidence of incapacity.

Clinical framework: Appelbaum PS, Grisso T. N Engl J Med 1988;319:1635–1638. doi:10.1056/NEJM198812223192504. Speech difficulty or aphasia alone is not incapacity — provide alternative communication. In Bulgarian law it guides the physician's assessment; it is not a separate statutory test.

First make the decision possible

treat pain and delirium; hearing aid, glasses, interpreter; a quiet setting and time; split complex information; repeat later; involve a person the patient trusts; consult a psychiatrist when in doubt.

Who consents? Health Act, Art. 87

Situation	Who consents
Adult with capacity	the patient alone (Art. 87(1)) — relatives have **no** right of veto
Minor 14–18; person under limited interdiction	the patient **and** a parent or curator (Art. 87(2))
From 16: counselling, preventive examinations, tests listed in Ordinance No. 8/2018	the minor alone (Art. 87(3))
Child under 14; person under full interdiction	a parent or guardian, except where the law provides otherwise (Art. 87(4))
Child placed outside the family by a court	if the parent's or guardian's consent cannot be obtained in time: the carer, after a positive opinion of the Social Assistance Directorate (Art. 87(5))
Child placed temporarily by administrative order	the Social Assistance Directorate (Art. 87(6))
Mental disorder with incapacity established by a court	the person appointed by the court in compulsory-treatment proceedings (Arts 87(7), 162(3)) — a special procedure, not a general route for every adult who cannot decide; for high-risk interventions also permission of the medical ethics committee — or of the head where there is none — and the representatives' consent (Art. 89(3))

A **relative** of an adult is not a legal representative: he informs the team about the patient's values but does not consent on his behalf. Whoever consents, the patient is involved as far as his capacity allows (Oviedo, Art. 6).

Minors aged 16 to under 18: what they may decide alone

Ordinance No. 8 of 27.06.2018 (issued under Health Act, Art. 87(3)) — without parental consent from age 16:

- **counselling** (Art. 2): behavioural health risk, mental health, **sexual and reproductive health**, dental health, health literacy;
- **preventive examinations** (Art. 3), incl. dental and specialist preventive examination;
- **tests** (Art. 4): HIV, syphilis, gonorrhoea, chlamydia, preventive blood and urine tests, **pregnancy test**.

> *Where a deviation of the health status or a disease is found during the consultations, examinations or tests, the physician who found it must notify the parent or curator in good time.*
>
> — Ordinance No. 8/2018, Art. 5

Outside the Ordinance

Treatment, prescriptions and invasive procedures for a minor require **both** his consent and a parent's (Art. 87(2)). Since 22.12.2023 nobody under 18 can marry in Bulgaria (Family Code, Art. 6(1)).

Case 5 · The parents are not there

The case

During a break a 10-year-old boy runs through a glass door at school and severs his radial artery. His teacher brings him to the emergency department. He is bleeding heavily and needs a transfusion and emergency vascular surgery. The parents cannot be reached by phone.

1. Is informed consent required here — and from whom?
2. May the teacher sign the consent?
3. What must the team do while operating, and what must it document?

Case 5 · The parents are not there — key points

Law

- For a child under 14 the parent consents (Art. 87(4)); surgery requires the written form (Art. 89(1)).
- Art. 89(2) and the Blood, Blood Donation and Blood Transfusion Act, Art. 32(1): when **life is threatened** and the parent's consent **cannot be obtained in time**, surgery and transfusion proceed without consent; the decision and reasons are recorded (Art. 32(2)).
- The teacher is **not** a legal representative; her signature has no legal value.

Practice

- Do not delay surgery to find the parents — but keep trying to reach them in parallel.
- Document: the threat to life, the attempts to contact the parents, the time, who decided.
- Inform the parents as soon as possible; explain to the boy, in a way suited to his age, what is happening.

Case 6 · Counselling at sixteen

The case

A 16-year-old girl comes alone to a gynaecologist for a preventive examination and asks for information about contraception. At the end she asks whether an intrauterine device can be inserted today and whether her parents "will find out". She is healthy; there is no emergency.

1. Can the physician examine and counsel her without a parent?
2. Do the conversation and the IUD insertion follow the same rule?
3. What can be promised about confidentiality — and what not?

Case 6 · Counselling at sixteen — key points

Law

- From 16, **preventive examination** and **counselling on sexual and reproductive health** need no parental consent (Art. 87(3); Ordinance No. 8/2018, Arts 2–3); a pregnancy test is included (Art. 4).
- Inserting an IUD is a **medical intervention** outside the Ordinance: her consent **and** a parent's are required (Art. 87(2)).
- If a disease or abnormality is found, the physician must notify the parent (Ordinance, Art. 5).

Ethics and communication

- Explain the **limits** of confidentiality at the start — do not promise absolute secrecy.
- Offer support for talking to a parent; give accurate information about methods, including condoms against infections.
- Check, tactfully, whether the relationship is voluntary and free of exploitation (Child Protection Act, Art. 7: duty to report a child at risk).

Case 7 · Almost of age

The case

A 17-year-old boy sees a urologist for phimosis. He asks for a circumcision but does not want his parents to know because he is ashamed. The procedure is simple and can be done under local anaesthesia. He will turn 18 in five months.

1. Can the urologist perform the procedure with the boy's consent only?
2. Does it matter that the intervention is minor and the boy is nearly 18?
3. How should the urologist respect his wish for privacy?

Case 7 · Almost of age — key points

Law

- Circumcision is a surgical intervention, not counselling or a preventive examination — Ordinance No. 8/2018 does not apply.
- At 17 both the boy's consent **and** a parent's are required (Art. 87(2)); written form applies to surgery (Art. 89(1)).
- No emergency: Art. 89(2) is irrelevant. The age of 18 is a fixed legal line, not a matter of degree.

Ethics and communication

- His consent is **legally necessary**, not mere "assent" — he is a co-decider.
- Options: help him talk to a parent; let him choose which parent; or postpone the elective procedure until he is 18, if medically safe.
- The examination and counselling themselves are confidential within the limits of the law.

Case 8 · The girl who lives with her partner

The case

A 17-year-old girl has lived with her 22-year-old partner in Plovdiv for six months after a religious ceremony; there is no civil marriage. She has found a lump in her left breast and asks for examination and removal. She has no contact with her parents, who work abroad. Her partner says he will "sign everything".

1. What can the physician do with her consent only?
2. Can the partner consent? Would a civil marriage change anything?
3. What must be done to reach the parents, and is there a child-protection concern?

Case 8 · The girl who lives with her partner — key points

Law

- Counselling within Ordinance No. 8/2018 may proceed. Diagnostic assessment of a **symptomatic** lump needs her consent **and** a parent's (Art. 87(2)); for invasive steps — biopsy, surgery — information and consent are written (Art. 89(1)).
- The partner has **no legal role**; since 22.12.2023 marriage below 18 is impossible (Family Code, Art. 6(1)).
- No emergency: contact the parents; the phone call supports the conversation, but identity and parental authority are verified and, for invasive steps, written consent is obtained and kept.

Ethics and protection

- Respect her autonomy and dignity — no judgement based on ethnicity or lifestyle.
- Assess **safety**: age gap, isolation from family, dependence. If she is at risk, the physician must notify the Social Assistance Directorate or the police (Child Protection Act, Art. 7(2)), even though bound by medical confidentiality.
- If parents are unreachable, the Social Assistance Directorate can help secure lawful consent.

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Informed refusal

> *The patient, respectively his parent, guardian or curator, ... may refuse at any time the proposed medical care or the continuation of the medical activity already started.*
>
> — Health Act, Art. 90(1)

- recorded in the medical documentation **with signatures** (Art. 90(2));
- if the person cannot or will not sign: **treating physician and a witness** sign (Art. 90(3));
- the refusal may be **withdrawn at any time**; the professionals are not liable for the resulting delay (Art. 90(5)).

> *When the patient, not knowing the dangerous consequences of the disease, refuses necessary tests or treatment, the physician must explain the unfavourable prognosis without that treatment in a way the patient understands — unless the patient expressly states that he does not wish to receive this information.*
>
> — Code of Professional Ethics, Art. 29 (abridged)

Refusal ends one intervention, not the physician's care: alternatives, palliation and the open door remain.

Two different legal situations: Art. 89(2) and Art. 90(4)

Health Act, Art. 89(2) — consent cannot be obtained	**Health Act, Art. 90(4) — consent has been refused**
life is **immediately** threatened **and** the patient's state does not allow him to express consent	the patient (or representative) has **refused** and the patient's life is threatened
and consent of a required representative cannot be obtained **in time**	the **head of the medical establishment** may decide on life-saving treatment
the team acts in the patient's health interest without written consent	a discretionary decision of a designated person — **not** automatic authority for every clinician
typical: unconscious trauma patient, child whose parents cannot be reached	typical: refusal of transfusion; parents refusing treatment of a child

Distinguish a parent's refusal, an adult's established autonomous refusal, and doubt about capacity or about whether a refusal applies. Art. 90(4) is read with Constitution Art. 52(4) and Oviedo Art. 5. For transfusion, the special Blood, Blood Donation and Blood Transfusion Act regulates consent (Art. 31), emergency (Art. 32) and refusal at any time (Art. 33) without providing for overriding a refusal — its relation to Art. 90(4) is a question of interpretation (general v. special law). ECtHR, Pindo Mulla v. Spain [GC], no. 15541/20, 17.09.2024 — Convention case law binding on Bulgaria too: violation of Art. 8 in the light of Art. 9; establish whether the refusal is free, informed, specific, current and applicable.

Treatment against the will and previously expressed wishes

> *Medical care against the will of the patient may be provided only in cases determined by law.*
>
> — Health Act, Art. 91

> *No one shall be subjected to forced medical treatment or sanitary measures except in cases provided for by law.*
>
> — Constitution, Art. 52(4)

Examples in the Health Act: **compulsory isolation** for listed infectious diseases (Art. 61); **emergency psychiatric help** when there is a direct and immediate danger (Art. 153); temporary placement after the emergency, up to 24 hours, extendable once by 48 hours with a judge's permission (Art. 154); **compulsory placement and treatment** by **court decision** only for persons under Art. 146(1)(1)–(2) who, because of the illness, may commit a crime dangerous to others or seriously endanger their own health (Arts 155–156). Compulsory isolation is not a general authority for any treatment against the will.

> *The previously expressed wishes relating to a medical intervention by a patient who is not, at the time of the intervention, in a state to express his or her wishes shall be taken into account.*
>
> — Oviedo Convention, Art. 9

In practice

Bulgarian law has no general statute on advance directives. Earlier wishes — oral or written, reported by relatives or in the record — are **evidence of the patient's values** that must be taken into account. Distinguish a general preference from a specific, applicable refusal.

Small-group work: cases 9–12

Task — 6 minutes

For your case answer:

1. Does the patient have decision-making capacity? How do you know?
2. Who decides — and on the basis of which article?
3. Is this Art. 89(2), Art. 90(4) or neither?
4. What will you do **now**, and what will you document?

Report — 2 minutes per group

- Group 1: Case 9 · Against the will of the parents
- Group 2: Case 10 · The veteran
- Group 3: Case 11 · "Leave me without doctors"
- Group 4: Case 12 · Who should decide?

Case 9 · Against the will of the parents

The case

A 10-year-old boy is admitted to haematology with severe anaemia; his condition is serious and he needs a blood transfusion. His parents are Jehovah's Witnesses and categorically refuse: they want to sign a refusal and say that for them a transfusion is worse than the death of their son. Alternatives have been tried without sufficient effect.

1. Can the parents' refusal be respected when it endangers the child's life?
2. Who decides in the hospital, and on what legal basis?
3. What duty arises under child-protection law?

Case 9 · Against the will of the parents — key points

Law

- The parents consent for a child under 14 (Art. 87(4)) and may refuse (Art. 90(1)); the refusal is documented (Art. 90(2)–(3)).
- With the child's **life threatened**, the **head of the establishment** may decide on life-saving treatment (Art. 90(4)); the special Blood Act (Arts 31–33) governs consent and refusal by the parent as legal representative. The decision is documented and taken in time; deterioration does not by itself cancel the refusal.
- A child at risk: duty to notify the Social Assistance Directorate (Child Protection Act, Art. 7).

Ethics

- Parental authority must be exercised for the **child's** welfare; parents may refuse for themselves what they may not refuse for their child.
- Respect the family: use all blood-sparing alternatives, explain, involve a representative the family asks for, keep them with the child.
- Adult patient ≠ child: an adult with capacity may refuse for himself (see Class 1 worked example).

Case 10 · The veteran

The case

Mr T., 84, a war veteran with no living relatives, lives in a care home. He is physically frail and has mild cognitive decline. He develops gangrene of the foot with increasing signs of systemic infection that do not respond to treatment; without amputation, sepsis with a high risk of death is expected within days. He refuses: "My foot will get better on its own. I've seen much worse in the army!"

1. Does mild cognitive decline mean he lacks capacity?
2. What exactly must be assessed — and is his reason a "wrong" reason?
3. If he lacks capacity, who can decide, given that he has no relatives?

Case 10 · The veteran — key points

Assessment

- Diagnosis and age do **not** prove incapacity. Test the four abilities for **this** decision.
- "It will heal on its own" may be a **failure to appreciate** the facts (a capacity problem) — or a value choice ("I will not live without my leg"). Find out which: explain again, show the wound, ask what matters to him.
- Treat reversible factors (infection, delirium, pain); involve a psychiatrist or geriatrician.

Who decides

- **With capacity:** an informed refusal (Art. 90(1)–(3)); palliation and the open door remain. Because life is threatened, the head's power under Art. 90(4) is in play — see the tension on the 89(2)/90(4) slide.
- **Without capacity:** he has no representative; support recovery of capacity; for urgent life-threatening deterioration the specific legal basis must be identified and documented; for non-urgent decisions obtain legal advice on representation.
- Never amputate merely because the refusal seems unwise.

Case 11 · Leave me without doctors

The case

A 68-year-old man deliberately drives his car into a pole in a suicide attempt. He arrives with multiple injuries and internal bleeding. In the emergency department he refuses the surgery needed to stop the bleeding: "I want to die." He offers to sign a refusal.

1. Is this an autonomous refusal that must be respected?
2. Does he have decision-making capacity at this moment?
3. What should the team do — and on what legal basis?

Case 11 · Leave me without doctors — key points

Assessment

- Suicidal intent raises **urgent concern** about capacity and voluntariness — but does not by itself establish incapacity.
- Assess rapidly how mental disorder, intoxication, head injury, shock and blood loss affect **this** decision; document it.
- If impaired autonomy is established, protecting him is **soft paternalism**, not overriding an autonomous choice.

Legal route

- Refusal + threat to life → the **head of the establishment** may decide on life-saving surgery (Art. 90(4)); losing consciousness does not by itself cancel the refusal.
- Psychiatric care: emergency psychiatric help (Art. 153), temporary placement after the emergency (Art. 154), compulsory treatment only by court on the criteria of Art. 155 — none of these alone authorises surgery.
- Document the refusal (Art. 90(2)–(3)), the capacity assessment and who decided.

Case 12 · Who should decide?

The case

A 45-year-old man is in coma in intensive care after a car crash, with a severe thigh injury. Two options: amputation, with a higher probability of survival but loss of the limb and subsequent functional limitation, or complex reconstruction with a higher risk of death but a chance to keep the leg. Delaying the decision by more than a few hours also increases the risk. His parents want amputation; his wife wants "everything possible to save the leg" and says she knows him better.

1. Who has the legal right to decide for an unconscious adult?
2. What weight do the relatives' views have?
3. How should the team reach and document the decision?

Case 12 · Who should decide? — key points

Law

- An adult without interdiction has **no legal representative**; neither the wife nor the parents can consent for him (Art. 87).
- With life immediately threatened and the patient unable to consent, the team decides in his health interest (Art. 89(2)); if there is time, delay the irreversible choice.
- Oviedo Art. 9: his **previously expressed wishes** must be taken into account.

Ethics and process

- Relatives are **witnesses of his values**, not decision-makers; the legal basis is established separately from the ethical reasoning: what would **he** want — work, sport, attitude to disability and risk?
- Medical judgement on risk and benefit; a **consultation among physicians** (consilium, Code, Art. 38) supports the judgement but creates no consent authority; transparency with both sides of the family.
- Document the reasons, the information gathered about his wishes and who took part.

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1. Concept and ethical foundations
2. Information, form and scope
3. Capacity, minors and representation
4. Refusal, emergency and treatment against the will
5. **Practice: documentation and responsibility**

Documentation, responsibility and research

What the record should show

- what was proposed; benefits, risks, alternatives, consequences of refusal;
- what support was given (interpreter, time, aids) and how understanding was checked;
- what the patient chose, with which limits; who was present; date;
- capacity assessment when there was doubt.

Revisit consent when

- the intervention or its scope changes;
- new information about risks or alternatives appears;
- the patient's condition, capacity or preferences change;
- a long time has passed before a planned intervention.

Responsibility

The duty to inform is the treating physician's; the team supports it. Consent never covers negligence.

Care is not research

Medical experiments require voluntary **written** consent (Constitution, Art. 29(2); Health Act, Art. 199(1)); ethics review and the different research regimes — Class 6.

Summary

Ethics

1. Consent is a **process**: information, understanding, capacity, voluntariness, a specific decision.
2. Capacity is functional and decision-specific; legal capacity is a status — a diagnosis does not change it, interdiction is decided by a court.
3. Refusal deserves the same care as consent: explain, document, keep the door open.

Bulgarian law

1. Content: Health Act Art. 88; written form: Art. 89(1); right not to know: Art. 92(2).
2. Who consents: Art. 87 — minors 14–18 co-decide with a parent; from 16, Ordinance No. 8/2018 activities alone.
3. Art. 89(2) (consent **cannot** be obtained) $\neq$ Art. 90(4) (consent **refused**; the head may decide).

Before every intervention: Who decides? Does he understand? Is the choice free? What exactly is authorised? How is it recorded?

Thank you for your attention!